DIGITALAIKART ORIGINAL

The Indian Dental Care & Cost Playbook

2026-27 EDITION

What a dental bill is really made of,
how to decide before you agree,
and how to stop paying for the same tooth twice

33 chapters · 5 printable worksheets · 2026 price tables for 5 cities
CGHS, ESIC and Ayushman Bharat benchmarks · 2020 IADT emergency protocols

Prices verified against the CGHS procedure rate list revised 13 October 2025
and published 2026 clinic price lists in Delhi NCR, Hyderabad, Lucknow, Indore and Patna

Before you begin

Three pages of orientation, and the one disclaimer that matters

This is a playbook about two things that are tangled together in India: what is wrong with your teeth, and what it will cost to fix them. Most dental writing picks one. This one refuses to, because the second question is where Indian families lose the most money and the first question is where they lose the most teeth.

It is written for the person sitting in the chair, not for the clinician. Every chapter ends with something you can do, ask or refuse. The price tables use published 2026 clinic ranges across Indian cities and set them against the CGHS rate list the government revised on 13 October 2025, so you can see what the same procedure is worth to a buyer with real bargaining power.

Three things you should know about the numbers in here. Ranges are ranges: a molar root canal costs more than a front tooth one in every city in India, and no table can change that. Prices move, and the CGHS list is the fastest-moving official document in this book, so it is dated everywhere it appears. And where a source was uncertain, the text says so instead of choosing the tidier number.

- If you have pain right now, go to chapter 6 and then chapter 25. Do not start at chapter 1.
- If you have a quote in your hand, go to chapters 2, 11, 12 and 30. That is forty minutes of reading and it is the highest-return forty minutes in the book.
- If nothing is wrong and you want to keep it that way, part three is the whole game: the daily routine, sugar frequency, children, and gum disease.
- If someone in the family has a knocked-out tooth, read chapter 24 today and save the box to your phone.

> **What this book is not**
>
> It is not a diagnosis and it is not a substitute for a registered dentist who has examined you. It cannot see your mouth, your X-ray or your medical history. What it can do is make you the kind of patient who gets shown the X-ray, gets the plan in writing, and does not pay Rs 22,000 for a crown on a tooth nobody sees. That patient gets better care, not just cheaper care.

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Five parts, thirty-three chapters, five worksheets

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PART ONE

The money

What a dental bill is made of, what the government already pays for, and five ways to pay less without getting worse care

CHAPTER 01

The same root canal, quoted at Rs 4,000 and Rs 14,000

Two clinics, one city, one tooth. The gap is not greed. It is four things you can see on a quote if you know where to look.

A root canal on a lower molar costs between Rs 5,000 and Rs 12,000 in a Delhi clinic in 2026, and the crown that goes on top afterwards costs between Rs 3,000 and Rs 22,000 depending on the material. Read that again: on the same tooth, the crown can cost more than the treatment that saved it, and most patients only hear the first number.

This is the single most useful thing to understand before you sit in a dental chair in India. A dental quote is not one price. It is a stack of line items, and clinics differ in which lines they put in front of you.

What is actually inside the number

- Which tooth. A front tooth has one canal and a short treatment. A molar has three or four, sometimes curved, and takes two to three times as long. The price difference is clinical, not cosmetic.
- How many visits. A single-sitting root canal is cheaper to deliver and usually fine for a tooth without active infection. An infected tooth needs medicament between visits.
- Who does it. A general dentist and a specialist endodontist charge differently for the same tooth, and the specialist is the right call for a re-treatment or a curved canal.
- What is used. Rotary files, a rubber dam, magnification and an apex locator all add cost and all reduce the chance of a failed root canal.
- What is excluded. The crown, the post, the core build-up, the X-ray and the CBCT are usually priced separately. A cheap headline number often has four add-ons behind it.

One more thing drives the gap, and it is not in the quote: the market. India has 329 dental colleges offering about 27,900 BDS seats a year, and 82 per cent of those seats are in private institutions (Observer Research Foundation, November 2025). The Dental Council of India registry counted 376,721 registered dentists in 2025, roughly one for every 3,846 people, which is about twice the density the World Health Organization recommends for developing countries. In a metro, several clinics compete on the same street. In a district town, there may be two. The same filling is priced by competition, not by difficulty.

The four-line test

Before you agree to anything above Rs 5,000, ask the clinic to write these four lines on paper. A clinic that cannot is a clinic that will surprise you later.

Line	What it must say	Why it matters
Procedure	Exact tooth number and exact procedure	Stops a second charge for the same tooth
Inclusions	Crown, post, X-ray, medicines, review visits	This is where Rs 3,000 becomes Rs 9,000
Material	Brand or grade of crown or implant, not just the word 'zirconia'	Rs 6,000 and Rs 22,000 crowns are both called zirconia
Warranty	What is covered, for how long, and what voids it	A one-year crown warranty is normal; a lifetime one is marketing

The one number to hold in your head

For any single tooth, a fair all-in private price in a metro in 2026 is roughly Rs 9,000 to Rs 25,000 for root canal plus a metal-ceramic or zirconia crown, and Rs 28,000 to Rs 75,000 for an implant with its crown. Below the bottom of that range, ask what is missing. Above the top, ask what extra you are buying.

CHAPTER 02

The 2026 price tables, city by city

Published clinic ranges for metros and tier-2 cities, set against the CGHS benchmark the government uses to buy the same procedures.

The table below is built from published 2026 price lists of private clinics in Delhi NCR, Hyderabad, Lucknow, Indore and Patna, plus a national range page that aggregates clinic data. These are indicative ranges, not quotes: your tooth, your city and your clinic decide the final number.

The third column is the interesting one. It is the Central Government Health Scheme rate list revised on 13 October 2025 and effective for CGHS empanelled centres. It is what the government pays a NABH-accredited hospital for the same procedure. Read it as the floor of what the procedure is worth, not as a price you can walk in and demand.

Procedure	Metro private	Tier-2 private	CGHS 2025 (NABH)
Consultation	Rs 300 - 1,000	Rs 200 - 500	Rs 350 (OPD)
Scaling and polishing	Rs 800 - 2,500	Rs 800 - 1,500	Rs 1,000
Deep cleaning, per arch	Rs 1,500 - 8,000	Rs 1,000 - 4,000	Rs 2,000
Composite filling	Rs 800 - 3,200	Rs 500 - 1,500	Rs 500 - 1,000
Root canal, front tooth	Rs 3,000 - 7,000	Rs 2,500 - 4,500	Rs 2,000
Root canal, molar	Rs 5,000 - 12,000	Rs 3,500 - 9,000	Rs 3,000
Root canal re-treatment	Rs 8,000 - 18,000	Rs 5,000 - 12,000	Rs 2,500 - 3,000
Metal-ceramic crown	Rs 3,000 - 10,000	Rs 2,000 - 4,500	Rs 3,000
Zirconia or E-max crown	Rs 6,000 - 22,000	Rs 5,000 - 9,500	Not listed
Simple extraction	Rs 500 - 2,500	Rs 300 - 1,500	Rs 400
Impacted wisdom tooth	Rs 2,500 - 18,000	Rs 2,000 - 8,000	Rs 2,000 - 5,000
Single implant with crown	Rs 28,000 - 75,000	Rs 25,000 - 35,000	Not listed
Metal braces, full course	Rs 35,000 - 70,000	Rs 20,000 - 40,000	Rs 15,000 per arch
Ceramic braces	Rs 50,000 - 90,000	Rs 30,000 - 45,000	Not listed
Clear aligners	Rs 90,000 - 3,50,000	Rs 45,000 - 1,50,000	Not listed
Complete denture, per arch	Rs 8,000 - 30,000	Rs 8,000 - 20,000	Rs 10,000
Veneer, per tooth	Rs 8,000 - 28,000	Rs 3,000 - 10,000	Not listed
OPG (full mouth X-ray)	Rs 300 - 1,500	Rs 300 - 800	Rs 500
CBCT (3D scan)	Rs 1,500 - 6,000	Rs 1,500 - 3,000	Rs 1,500
Fluoride varnish, child	Rs 500 - 3,000	Rs 300 - 1,500	Rs 1,000

Sources: published 2026 clinic price lists for Delhi NCR, Hyderabad, Lucknow, Indore and Patna; CGHS procedure rate list revised 13 October 2025, Tier I. Where the CGHS list has no entry, the row says so, and that absence is itself information.

Three things the table tells you

- Metro and tier-2 prices differ by 40 to 60 per cent on the same procedure. If a treatment is not urgent, a planned trip home can pay for itself on a single implant.
- The CGHS benchmark sits far below private prices for basic work and close to it for prosthetics. That is not a coincidence: the government buys crowns and dentures from a competitive lab market, but pays a fixed professional fee for time.

- Implants and aligners have no CGHS line at all. Those two are the products India's public system does not buy, which is exactly why they are marketed hardest to you.

> **How to use the table honestly**
> Do not walk into a clinic and demand the CGHS rate. Use the table to ask a better question: 'Your quote is Rs 14,000 for this molar. The published range in this city is Rs 5,000 to Rs 12,000. What is in your number that is not in the others?' You will get either a clear answer or a discount.

CHAPTER 03

What the government already pays for

CGHS, ESIC, Ayushman Bharat, dental colleges and school programmes. For most Indian families, one of these is closer than they think.

The most common expensive mistake in Indian dentistry is paying private rates for something a scheme you already belong to would have covered. Start by finding out which of these you are inside.

CGHS: central government employees and pensioners

The Central Government Health Scheme runs empanelled private centres and its own wellness centres. The rate list revised on 13 October 2025 covers the full range of routine dentistry: scaling, fillings, root canals, crowns, dentures, extractions and even fixed orthodontics at Rs 15,000 per arch in a NABH-accredited centre. If you are a central government employee, a pensioner, a dependent, an MP, a freedom fighter or a journalist accredited with the Press Information Bureau, and you have a valid CGHS card, you can use this. The catch is that you must go to an empanelled centre and follow the referral process, and implants are simply not on the list.

ESIC: organised-sector workers earning up to the wage ceiling

Employees' State Insurance Corporation beneficiaries get dental care through ESIC dispensaries and tie-up hospitals. From 1 December 2025, ESIC adopted the CGHS rate list across its user locations, which means the same benchmarks now apply to ESIC empanelled care. If your employer deducts ESI, ask the dispensary what dental work they can do in-house before you pay retail.

Ayushman Bharat PM-JAY: Rs 5 lakh, but only for hospitalised oral surgery

This is where most people get it wrong in both directions. PM-JAY does not cover routine dentistry. The Health Benefit Package 2.2 manual, published by the National Health Authority, excludes, in its own words, 'any dental treatment or surgery which is corrective, cosmetic or of an aesthetic procedure, filling of cavity, root canal including wear and tear etc. unless arising from disease or injury and which requires hospitalization'. What it does cover is oral and maxillofacial surgery: jaw fractures, impacted tooth removal under anaesthesia, cysts and tumours of the jaw, cleft lip and palate repair, oral cancer surgery, and the release of fibrous bands in oral submucous fibrosis. Those are the cases where a dental problem becomes a hospital admission, and those are the ones where a family can be saved Rs 20,000 to Rs 2,00,000.

The WHO's 2022 country profile for India states the same thing in scheme language: routine and essential curative oral health care are inside the largest government health financing scheme, advanced curative care and rehabilitation are not.

Government dental colleges: the cheapest real option

India has 329 dental colleges. Fewer than one in five is in the public sector, and every one of them runs an outpatient department open to the public. Charges are nominal government rates, the work is done by supervised postgraduate or final-year students under faculty, and the materials are standard. What you pay in money, you pay back in time: waiting lists, multiple visits, and appointments that move. For a single filling, it is rarely worth the queue. For full dentures, multiple crowns, orthodontics or a full-mouth rehabilitation, the saving can run into tens of thousands of rupees.

The programmes almost nobody uses

- Primary health centres and community health centres in many states now have a dental unit under the National Oral Health Programme, launched in 2014. Extractions, scaling and basic fillings are available close to home.
- eDantSeva, the oral health portal run under the same programme, carries screening and awareness material and links to services.
- The Rashtriya Bal Swasthya Karyakram screens school children for dental conditions and refers them onward, including to District Early Intervention Centres.
- State schemes sometimes go further than PM-JAY. Chhattisgarh, for example, added a set of dental packages to its Ayushman implementation years ago, which no national scheme carries.

> **The 15-minute check worth doing this week**
> Find out, in writing, which of these you belong to: CGHS card, ESIC, PM-JAY family, state scheme, corporate group health policy with an OPD or dental benefit, or a government dental college within reach. Save the answer in your phone. The next time someone in the family has a toothache at 9 p.m., you will make a decision in minutes instead of paying whatever the nearest clinic quotes.

CHAPTER 04

Insurance: what Indian policies actually cover

Most retail health policies exclude teeth. Some group policies do not. Here is how to find out which one you hold, in ten minutes.

India's dental spending is almost entirely out of pocket. A 2026 systematic review of the economic burden of oral disease in India found insurance coverage for oral care below 15 per cent, catastrophic health expenditure on dental care pooled at 18.8 per cent of households using the 20 per cent-of-income threshold, and up to one-third of patients financing treatment by borrowing or selling assets. The WHO country profile puts India's per-person spending on dental healthcare at five US cents a year.

So the honest starting point: do not assume your policy will pay. But do check, because the exceptions matter.

The four places dental cover hides

- Group health policies from employers. Many corporate floater policies include a dental sub-limit, an OPD wallet, or cover for dental procedures that need hospitalisation. Ask HR for the policy wording, not the summary brochure.
- Accident cover. Dental treatment following an injury, including a knocked-out or fractured tooth, is usually payable under the accident section of a personal accident or health policy. A road accident that breaks teeth is a claim, not a cash payment.
- Hospitalisation-linked dental surgery. Impacted wisdom tooth removal under general anaesthesia, jaw fracture fixation, cyst removal and orthognathic surgery are inpatient procedures, and inpatient procedures are what indemnity policies are built for. Get pre-authorisation before admission, not after.
- Newer OPD-style products. Some policies sold since 2023 bundle outpatient consultations, diagnostics and dental cleaning into a wallet. Read the sub-limits: a Rs 5,000 dental wallet with a Rs 500 per-visit cap is not cover, it is a coupon.

The three sentences to look for in your policy wording

Open the exclusions section and search for the word 'dental'. You will usually find a clause close to this: dental treatment or surgery of any kind unless requiring hospitalisation due to an accident or disease. The words 'unless requiring hospitalisation' are the whole game. If the treatment can be done in a chair and you walk out, it is excluded. If it needs a bed, an anaesthetist and a day-care or inpatient admission, it is often payable.

Second sentence: waiting periods. Dental claims arising from an accident usually have no waiting period; disease-linked dental surgery may fall under the general 30-day or 24-month clauses.

Third sentence: pre-authorisation. For any planned dental surgery, call the insurer's helpline before admission and get the pre-auth number in writing. A claim rejected for a missing pre-auth is the most common avoidable rejection

in Indian health insurance.

> **What to do with this chapter**
>
> This week, message HR one question: 'Does our group policy cover dental treatment, and what is the sub-limit and claim process?' If the answer is yes, ask for the empanelled dental clinics in your city. Corporate dental networks often bill at negotiated rates 20 to 30 per cent below walk-in prices even before the insurer pays anything.

CHAPTER 05

Five ways to pay less without getting worse care

Ranked by how much they save, with the trade-off attached to each one.

Cheap dentistry and inexpensive dentistry are not the same thing. The five routes below lower the price without lowering the standard of care. They lower something else instead: your time, your travel, or your choice of who holds the instrument.

Route	Typical saving	What it costs you
Government dental college or hospital dental department	60 - 85%	Waiting lists, student operators, multiple visits, travel
CGHS or ESIC empanelled centre, if eligible	40 - 70%	Referral paperwork, limited material choices
Corporate or insurer network rate	15 - 30%	Must use listed clinics, claim process
Second opinion before agreeing	20 - 100% of the plan	One extra consultation fee, one week
Material and staging choices	10 - 40%	Fewer aesthetic options, longer total timeline

1. Use the teaching hospital for the big-ticket work

A single filling: go private, the queue is not worth it. Full dentures, four crowns, a full-mouth rehabilitation, or orthodontics for a teenager: the arithmetic flips completely. A Rs 1,20,000 rehabilitation at a private clinic can be a fraction of that in a government college, with the same materials. Book the first appointment in a quiet month, keep every appointment, and ask for the faculty supervisor at each stage.

2. Check the panel before the pain

Find out now whether your CGHS card or ESIC entitlement covers the clinic ten minutes from your house. Ten minutes of paperwork on a Sunday beats an argument at a reception desk on a Tuesday with a swollen jaw.

3. Ask for the network rate even without a claim

Clinics that sit on insurer and corporate panels often have a cash rate close to the network rate. Ask: 'Do you bill insurers? What is your cash price at the network rate?' It is a normal question and it is usually answered.

4. Buy a second opinion, not a second treatment

A second consultation costs Rs 300 to Rs 1,000 and an OPG film. Against a plan worth Rs 40,000, that is the best-value purchase in this book. Chapter 13 gives you the script so it does not feel like an accusation.

5. Choose materials deliberately, not by default

A metal-ceramic crown on a back molar does the job a zirconia crown does, for a third of the price, and the only person who sees it is your tongue. Spend the aesthetic money on the teeth people actually see. Similarly, a removable partial denture at Rs 2,000 to Rs 8,000 is a legitimate solution for some patients and a bad one for others; chapter 9 sets out who is who.

The rule that saves the most money

Treat the quote as a draft, not a verdict. In every large Indian city, a treatment plan above Rs 20,000 is negotiable on scope and timing, and almost never negotiable on the clinical diagnosis. Change what is done and when it is done. Never bargain on whether the infection is removed.

PART TWO

Decide before you agree

Reading your own symptoms, choosing between procedures, and the questions that separate a treatment plan from a sales plan

CHAPTER 06

Read your own symptom before anyone reads your X-ray

A decoder for the eight things patients actually feel, with the urgency attached to each.

Dentistry is unusual in medicine: most of what goes wrong announces itself early and cheaply. The problem is that patients cannot tell the Rs 800 problem from the Rs 40,000 problem, so they either ignore both or panic about both. This table closes that gap.

What you feel	What it usually means	How urgent
Sharp pain to cold that stops in a second or two	Reversible sensitivity, exposed root, small cavity	Book within 2-3 weeks
Pain to cold or hot that lingers 30 seconds or more	The nerve is inflamed; irreversible pulpitis	Book within days
Spontaneous throbbing at night, wakes you up	Advanced pulpitis or early infection	Same week
Pain on biting one specific tooth	Cracked tooth, high filling, or infection at the root tip	Within days
Gums bleed when brushing	Gingivitis; the earliest, cheapest stage	Book a cleaning within a month
Loose tooth in an adult	Periodontal bone loss until proven otherwise	Within a week
Swelling of the face or gum with fever	Spreading infection	Today
Swelling that reaches the eye or the floor of the mouth, or you cannot swallow	Deep space infection	Emergency room, now

The three symptoms people get wrong most often

- Bleeding gums are not caused by a hard brush. They are caused by plaque at the gum line. If your gums bleed, the brush is not the problem; the plaque is. Brushing more carefully makes it worse; cleaning the gum line properly makes it stop within two weeks.
- A tooth that no longer hurts is not a tooth that healed. Pain stops when the nerve dies, not when the infection leaves. A dead tooth with a silent abscess can destroy bone for years and is exactly how a Rs 6,000 root canal becomes a Rs 60,000 implant.
- Sensitivity after a cleaning or a filling is normal for a few days. Sensitivity that starts two weeks later, or wakes you at night, is not.

> **Photograph it**
> Before you go to any clinic, take two photos on your phone: one with the mouth open showing the tooth, and one from the side in natural light. Swelling and asymmetry are obvious in a photo and hard to remember. If the problem escalates, you have a baseline.

CHAPTER 07

Filling, root canal or extraction

The decision that costs the most money and gets made fastest. Here is the logic a careful dentist uses.

Three procedures, three price bands, one question behind all of them: is the nerve of the tooth still alive and is the tooth still strong enough to keep?

Situation	Right answer, usually	Why
Cavity in enamel or dentine, no nerve symptoms	Filling	Cheapest, fastest, and stops the process
Cavity reaching the nerve, or lingering pain to heat	Root canal plus crown	Removes infected tissue, keeps the root

Situation	Right answer, usually	Why
Root-treated tooth with heavy loss of structure	Crown after the root canal	A root-treated molar without a crown fractures more often
Vertical root fracture, or tooth split below the gum	Extraction	No procedure can save a split root
Advanced gum disease with mobility and bone loss	Extraction, sometimes	A root canal cannot rebuild lost bone
Deep infection with a large abscess and no restorable tooth	Extraction plus drainage	Saving the tooth would mean repeated failure

How long does a root canal actually last

In published reviews, roughly nine out of ten root-treated teeth are still in function two to ten years later. The failures cluster around three causes: a missed canal, a leaking restoration, and a tooth that was never crowned afterwards. Which is why the crown is not an upsell, and why a root canal quoted at Rs 3,500 with no crown in the plan is an incomplete plan, not a bargain.

When extraction is the right answer

A dentist who recommends extraction too easily is a problem. So is a dentist who will root-canal anything. The honest signals for extraction are specific: a vertical root fracture, a root that is perforated, a tooth with no bone support, or a tooth whose root canal has failed twice with a persistent infection. In those cases, spending Rs 15,000 on a re-treatment buys time, not a fix, and the implant you eventually need costs more because the bone has gone.

> **The question that changes the plan**
>
> Ask: 'If this were your own tooth, would you root canal it or take it out, and what would you do about the crown?' A dentist who answers in one sentence without hedging is telling you something. So is one who says 'we will decide after we open it' before any X-ray has been shown to you.

CHAPTER 08

Why the crown costs more than the root canal

Five crown materials, what each one is actually for, and the one question that decides between them.

A crown is a cap that covers a weakened tooth. It is made in a dental laboratory from a scan or an impression, and the lab bill is a large part of what you pay. That is why the material drives the price more than the dentist's time does.

Material	Private range, 2026	CGHS benchmark	Best used for
PMMA or acrylic (temporary)	Rs 800 - 2,000	Rs 1,000	Short-term cover while a final crown is made
Full metal (nickel-free)	Rs 1,700 - 6,000	Rs 2,000	Back molars where strength beats looks
Metal-ceramic (PFM)	Rs 3,000 - 10,000	Rs 3,000	The workhorse: strong, acceptable looks, moderate price
Zirconia	Rs 6,000 - 22,000	Not listed	Back teeth needing strength, or patients who grind
E-max / lithium disilicate	Rs 9,000 - 22,000	Not listed	Front teeth where appearance decides

The CGHS column is instructive: the government buys a temporary crown, a metal crown and a metal-ceramic crown, and stops there. Zirconia and E-max are private-market products. Neither is a scam; both are genuinely better in specific situations. But if a clinic quotes Rs 20,000 for a zirconia crown on a lower molar and cannot explain what the zirconia does that a Rs 4,000 metal-ceramic crown does not, you are paying for a brochure.

The two questions that decide the material

- Is this tooth visible when you smile? If no, the aesthetic premium is buying nothing. If yes, it is buying something real: metal-ceramic crowns show a grey line at the gum after a few years.
- Do you grind or clench at night? If yes, ask about a night guard, which the CGHS list prices at Rs 1,700 to Rs 2,000, before spending Rs 20,000 on a crown that will chip.

When a crown is not needed

A crown is indicated when the tooth has lost too much structure for a filling to hold, or after a root canal on a back tooth. It is not indicated for a small cavity, for a tooth that is merely sensitive, or as a substitute for treating gum disease. If a plan lists eight or ten crowns at once, ask for the X-rays and get a second opinion before anything is prepared. Once a tooth is cut down for a crown, it is cut down permanently.

> **One line to say in the chair**
> 'Before we prepare the tooth, can I see the X-ray that shows why a crown is needed rather than a filling?' It is a completely reasonable request. A clinic that resists it is answering your question.

CHAPTER 09

One missing tooth, three ways to fill the gap

A ten-year cost comparison of doing nothing, a partial denture, a bridge and an implant.

A missing molar looks like a cosmetic problem. It is a structural one. The tooth behind it tilts forward, the tooth above it over-erupts, the bone under the gap shrinks, and the bite slowly redistributes load onto fewer teeth. None of that happens in a month, and all of it is expensive later.

Option	Upfront cost	Ten-year view	Main downside
Do nothing	Rs 0	Cheapest today, most expensive later	Bone loss, drifting teeth, more complex treatment
Removable partial denture	Rs 2,000 - 8,000	Rs 2,000 - 16,000 with relines and replacements	Bulk, movement while eating, clasp wear on neighbours
Bridge (three units)	Rs 5,000 - 30,000	Rs 5,000 - 30,000 unless a support tooth fails	Two healthy teeth are cut down permanently
Implant with crown	Rs 25,000 - 75,000	Rs 25,000 - 75,000; roughly 95 in 100 still in place at ten years	Needs bone, surgery, 3-4 months, no CGHS listing

The implant figure comes from systematic reviews of single implants, which consistently report ten-year survival around 94 to 96 per cent. That is not a promise about your mouth; it is the average across thousands of patients, and it depends on bone, hygiene, smoking and who placed it.

The decision that actually matters

- If the gap is at the back and nobody sees it, a removable denture is a legitimate long-term answer for many patients, especially after 60.
- If the neighbouring teeth are already heavily filled and need crowns anyway, a bridge converts work you need into a solution you need. That is the one situation where a bridge is clearly smart.
- If the neighbouring teeth are healthy, a bridge means cutting two good teeth to replace one. Most dentists would not do that to their own mouth today.
- If you are under 45, have good bone, do not smoke, and the gap is visible or load-bearing, an implant is usually the option that costs least over a lifetime, even though it costs most on the day.

The bone clock

After a tooth is lost, the bone in that area starts to shrink, most in the first year. That is why an implant placed within a few months of extraction is often simpler and cheaper than one placed five years later, when a bone graft at Rs 5,000 to Rs 30,000 may be needed first. If you are unsure, get the implant assessment early even if you decide to wait.

CHAPTER 10

Braces and aligners: who needs them, and when to start

Four bracket systems and a clear aligner market, priced honestly, plus the timing that avoids a second round.

Orthodontics is the most expensive routine dental purchase an Indian family makes, after implants. It is also the one where timing changes the result more than money does.

Option	Metro range	Tier-2 range	Notes
Metal braces	Rs 35,000 - 70,000	Rs 20,000 - 40,000	Most robust, least discreet, monthly adjustments
Ceramic braces	Rs 50,000 - 90,000	Rs 30,000 - 45,000	Tooth-coloured brackets, more fragile, staining risk
Self-ligating braces	Rs 60,000 - 1,40,000	Rs 35,000 - 70,000	Fewer visits, not automatically faster
Lingual braces	Rs 1,60,000 - 3,20,000	Rs 60,000 - 1,50,000	Invisible, hardest to clean, specialist skill
Clear aligners	Rs 90,000 - 3,50,000	Rs 45,000 - 1,50,000	Mild to moderate cases, 22 hours a day of wear

Who actually needs treatment

Orthodontic treatment is worth it when it changes function or long-term tooth survival, not only appearance. Crowding that makes cleaning impossible, a deep bite that wears the front teeth, a crossbite, an open bite that prevents chewing, and protruding upper teeth that fracture in falls are all functional problems. Mild irregularity in a mouth with healthy gums is a preference, and a legitimate one, but it should be bought as a preference, not sold as a necessity.

The timing that matters

- First orthodontic assessment by age 7, when the first permanent molars and incisors are in. Most children need nothing at that point; the ones who need early intervention are identifiable, and early intervention is short and cheap.
- The main window for comprehensive treatment is roughly 10 to 14, when the growth spurt can be used to move jaws as well as teeth.
- Adults can be treated at any age, but treatment takes longer, and gum health has to be stable before a bracket goes on.
- Retention is not optional. Without a retainer, teeth drift back, and the retainer is a permanent cost, not a finishing touch.

The three questions before you pay for aligners

Is the treating dentist an orthodontist (MDS in orthodontics) or a general dentist running a scanner? Is the plan a full-course plan with retainers included? What happens, and who pays, if the teeth do not track and you need fixed braces midway? Ask all three before the scan, not after the payment.

CHAPTER 11

Twelve signs a treatment plan is too aggressive

Not accusations. Signals to slow down and ask one more question.

Most dentists in India do careful, honest work under difficult economics. But the incentives in a market where 82 per cent of dental education is private and clinics are dense are real, and a patient who can recognise a pattern protects themselves without insulting anyone.

1. A plan with more than four crowns or more than four root canals in one sitting, presented without X-rays you can see.
2. A root canal recommended on a tooth that has never hurt and shows no visible cavity, with no vitality test done.
3. Every treatment recommended as urgent today, with a discount that expires this week.
4. A quote with a single total figure and no itemisation.
5. A clinic that will not give you your X-rays. They are your records. Ask for a copy, and expect to get it.
6. A diagnosis delivered without any instrument in your mouth: no probe, no air, no cold test.
7. Extraction recommended for a tooth that another dentist said could be saved, without the reason being explained in clinical terms.
8. Full-mouth rehabilitation proposed to someone with no pain and no function complaint.
9. Implants offered without a CBCT or bone assessment, on the same visit as the consultation.
10. A 'free' check-up that ends with a Rs 30,000 plan and a same-day payment discount.
11. Whitening, veneers or a smile makeover offered to a patient with untreated gum disease. Fix the foundation first, always.
12. Any plan that does not mention prevention: no hygiene instruction, no cleaning interval, no discussion of what caused the problem.

> **What a good plan looks like**
>
> A diagnosis in plain words, an X-ray or photo on the screen in front of you, a written itemised plan with a sequence (urgent first, elective later), a statement of what happens if you do nothing, and a cost that stays the same unless something changes clinically. If any of those five is missing, you are allowed to ask for it.

CHAPTER 12

Fifteen questions to ask before you say yes

Print this page. Take it with you. Nobody will be offended.

1. What exactly is wrong with this tooth, and how can you tell?
2. Can I see the X-ray and what you are pointing at?
3. Is the nerve alive, and did you test it?
4. What happens if I wait three months?
5. What are my options, including the cheapest one?
6. What is included in this price, and what will be billed separately?
7. Which material are you using, and who makes it?
8. How many visits, and how long will each one take?
9. Who will do the work: you, a colleague, or a student?
10. What is the success rate of this procedure in your hands?
11. What are the risks, and what would a failure look like?
12. What is the warranty, in writing, and what voids it?

13 What should I do if it hurts at night or over the weekend?

14 What do I do at home so this does not happen again?

15 Can I have all of this in writing before we start?

You do not need to ask all fifteen. Ask four, five, six and fifteen. Those four questions alone change the conversation, because they move it from a price to a plan.

The single sentence that works

'I am not saying no. I would like the itemised plan in writing so I can arrange the money.' It buys you a week, costs nothing, and no reasonable clinic refuses it.

CHAPTER 13

Getting a second opinion without wrecking anything

Where to go, what to bring, what it costs, and the exact words to use.

A second opinion is normal medical practice, not an insult. In dentistry it is also cheap relative to what it protects. One consultation and one OPG film cost Rs 500 to Rs 2,000 against a plan that is often Rs 20,000 or more.

What to bring

- The written treatment plan and the itemised quote from the first clinic.
- A copy of every X-ray, especially the OPG. You are entitled to your own records.
- Your two phone photos, and a note of when the symptoms started and what makes them better or worse.
- A list of your medicines and conditions: diabetes, blood thinners, pregnancy, heart valve problems, bisphosphonates.

Where to go

Choose a clinic that is not in the same building and preferably not in the same referral network. A dental college OPD is an excellent second-opinion venue because the incentive structure is different: they are not selling you the treatment. A specialist (endodontist for a root canal question, periodontist for gum, orthodontist for braces) is worth the extra fee when the first plan is unusual.

The script

Say this

'I have a treatment plan from another clinic and I would like a second opinion before I start. I am not asking you to criticise them. I would like to know whether you would treat this the same way, and if not, what you would do differently and why.'

Then listen for two things. Does the second dentist examine you properly before answering? And does the answer explain itself in clinical terms rather than in price terms? If both dentists examine you, disagree on the diagnosis, and can each explain why, get a third opinion from a dental college. If they agree on the diagnosis and differ only on materials and cost, you have your answer and you can choose on price.

When you should not get a second opinion

Facial swelling with fever, swelling near the eye, difficulty swallowing or opening the mouth, uncontrolled bleeding after an extraction, or a knocked-out tooth. Those are treatment-now situations. Get treated, then review the plan afterwards.

PART THREE

Prevention that actually works

The daily routine, the family timeline, and the claims worth paying for and the ones worth ignoring

CHAPTER 14

The daily protocol that has evidence behind it

Four habits, in order of how much they matter. Everything else is optional.

Most dental disease in India is preventable, and the prevention is not exotic. It is fluoride, frequency, and cleaning between the teeth. What follows is the routine, ranked, with the reason each step exists.

1. Brush twice a day for two minutes, with fluoride toothpaste

The active ingredient is fluoride, not the brush. Look at the back of the tube for a number between 1,000 and 1,500 ppm. If the tube does not state the fluoride concentration, buy a different tube. Children under three need only a rice-grain smear; three to six need a pea-sized amount, and an adult should supervise so the paste is spat out, not swallowed.

Technique matters less than coverage. Angle the bristles at 45 degrees to the gum line (the modified Bass method), use small circles, and cover every surface: outside, inside, and the chewing surfaces. Two minutes is longer than it feels. Use a timer once and you will learn what two minutes actually is.

2. Spit, do not rinse

This is the single most commonly missed instruction in Indian dentistry. After brushing, spit out the excess paste and do not rinse with water or mouthwash. Rinsing washes away the fluoride film that is doing the work. If you must rinse, wait 30 minutes.

3. Clean between the teeth once a day

A toothbrush reaches about 60 per cent of each tooth surface. The rest sits between the teeth, which is exactly where adult cavities and gum disease start. Floss where the contacts are tight; interdental brushes where there are gaps or gum recession. For braces and implants, a water flosser earns its price.

4. Do not brush immediately after acid

After citrus, vinegar-based food, a cola, or vomiting, the enamel surface is temporarily softened. Brushing straight away abrades it. Rinse with plain water, wait 30 minutes, then brush. This is the single change that matters most for people with gastric reflux or morning sickness.

Habit	Evidence strength	Effect
Fluoride toothpaste, twice daily	Strong	The largest single reduction in cavities of any habit
Interdental cleaning daily	Strong	Reduces gum inflammation and interproximal decay
Spit, do not rinse	Moderate to strong	Keeps fluoride concentration up after brushing
Professional cleaning every 6-12 months	Moderate	Removes calculus a brush cannot touch
Fluoride varnish in children	Strong (Cochrane)	43% fewer decayed surfaces in permanent teeth
Fissure sealants on new molars	Moderate (Cochrane)	11-51% fewer cavities at 24 months

What to buy this month

A fluoride toothpaste at 1,000-1,500 ppm, a soft-bristled brush you replace every three months, and interdental brushes or floss in a size that fits your gaps. That is the whole shopping list. Everything else in the dental aisle is optional, and chapter 16 tells you which optional things are worth it.

CHAPTER 15

Sugar frequency beats sugar quantity

India makes 53.8 grams of sugar available per person per day. The damage is not the total. It is how many times a day your teeth meet it.

The WHO's 2022 country profile for India records per-capita sugar availability of 53.8 grams a day. That is the average, which means a large part of the population is well above it, and most of that excess arrives in liquid and snack form rather than in a spoon.

The 20-minute cycle

Every time sugar meets plaque bacteria, acid is produced and the pH at the tooth surface drops below the point where enamel starts to dissolve. It stays there for roughly 20 to 30 minutes, until saliva neutralises it. Five sweets eaten at one sitting cause one acid attack. One sweet eaten every hour for five hours causes five. This is why frequency, not total quantity, drives decay, and why a single chocolate bar is less harmful than a packet of biscuits nibbled across an afternoon.

The Indian plate, specifically

- Packaged fruit juice and health drinks. Liquid sugar has no chewing time and no fibre, and it bathes every surface. The label on a 200 ml carton usually prints the number, and it is rarely under 20 grams.
- Biscuits and rusks, the default Indian tea companion. Two biscuits, four times a day, is eight acid attacks, and the tea usually carries sugar too.
- Chikki, jaggery sweets and honey. Jaggery and honey are not protective. They are sugar with minerals attached, and they stick.
- Sticky dried fruit and sweets eaten slowly over hours at festivals. This is the pattern that produces a cluster of new cavities after Diwali.
- Fizzy drinks and energy drinks, including the 'zero sugar' ones. The carbonation and acid still erode enamel, even without sugar.

The five practical rules

1. Keep sugar to mealtimes. Three meals plus one snack is the target, not a rule of virtue.
2. Drink water after anything sweet. It shortens the acid attack without brushing softened enamel.
3. Cheese, milk, peanuts and plain curd at the end of a meal help neutralise acid. This is why a piece of cheese after dinner is a real dental intervention.
4. Chew sugar-free gum with xylitol for ten minutes after a meal when you cannot brush. The evidence for xylitol gum reducing cavities is moderate but consistent.
5. Read labels for added sugar, not for the word 'natural'. Fruit concentrate, maltodextrin, invert syrup and honey are all sugar to your plaque.

> **One change for a family with children**
>
> Move every sweet thing in the house to one designated time of day, after a meal, and replace the all-day biscuit tin with something that is not sticky. In households that do this, the number of daily acid attacks usually falls from six or more to two. That single change does more than any toothpaste upgrade.

CHAPTER 16

What works, what is harmless, and what is a waste of money

A verdict table for fourteen things Indians buy or are told to do, with the evidence behind each verdict.

Dental aisles and YouTube are full of advice. Some of it is good, some is harmless, and some is actively expensive because it substitutes for something that works. Here is the honest ranking.

Claim or product	Verdict	Why
Oil pulling with coconut or sesame oil	Skip	No reliable trial evidence it reduces cavities or gum disease; harmless but not a substitute for anything
Charcoal toothpaste	Skip	Abrasive, often fluoride-free, no evidence of whitening beyond stain removal
Salt water rinse	Harmless	Soothes sore gums after extraction; does not treat infection
Alcohol-based mouthwash daily	Avoid	Dry mouth, staining, and no benefit over brushing; use alcohol-free, short courses only
Chlorhexidine mouthwash	Use as prescribed	Effective for 7-14 days after surgery; long use stains teeth
Flossing daily	Works	Reduces interproximal decay and gum inflammation
Interdental brushes	Works better than floss in gaps	Designed for the space; easier to use correctly
Water flosser	Useful in specific cases	Best for braces, implants and dexterity problems
Tongue scraping	Harmless to mildly useful	Reduces coating and breath odour; no effect on decay
Miswak	Works	Mechanical cleaning plus naturally occurring fluoride; use it as a brush, not as a replacement for paste
Baking soda brushing	Avoid daily	Abrasive over years; fine occasionally
Whitening strips and LED kits	Limited, with risk	Can work on surface stains; gum burns and sensitivity are common; dentist-supervised is safer
Fluoride varnish in children	Works	43% reduction in decayed surfaces in permanent teeth (Cochrane)
Sugar-free xylitol gum	Works, modestly	Chewing after meals raises saliva flow and reduces decay

The two things that are not on this list

Professional scaling and a filling. Both are procedures, not products, and both are dramatically cheaper than the alternative. A Rs 1,000 cleaning every year is the cheapest dentistry in this book. A Rs 1,500 filling that stops a cavity is the second cheapest.

> **On whitening, honestly**
>
> No whitening product changes the colour of a filling, a crown or a veneer, and none of them fixes the cause of staining. If your teeth are yellow because of plaque, tobacco or coffee, a cleaning is the answer. If they are yellow because of the dentine underneath, whitening works, and it works better and more safely in a chair than in a bathroom.

CHAPTER 17

Bleeding gums: the disease that costs you teeth

Gum disease is more common than cavities in Indian adults, and it is the main reason people over 50 lose teeth.

A 2026 review of India's oral health system pooled the published data and found gingivitis in 46.6 per cent of the population and periodontitis in 51 per cent of adults aged 15 and over. The WHO's country profile puts severe periodontal disease at 21.8 per cent of people over 15. This is not a minority condition. It is the norm, and it is nearly silent until it is expensive.

How it progresses

- Plaque sits at the gum line. The gum becomes red, puffy and bleeds when brushed. This is gingivitis, and it is fully reversible in about two weeks with a professional cleaning and better daily cleaning.
- Plaque hardens into calculus, which cannot be brushed off. The gum detaches from the tooth and forms a pocket.
- The bone that holds the tooth begins to recede. This is periodontitis. Bone does not grow back.
- Teeth become mobile, gaps appear between front teeth, and the bite changes. At this stage the choice is management, not cure.

What treatment involves, and what it costs

Stage	Treatment	2026 private range	CGHS benchmark
Gingivitis	Scaling and polishing, 1 visit	Rs 800 - 2,500	Rs 1,000
Early periodontitis	Scaling and root planing, per arch	Rs 1,500 - 8,000	Rs 2,000
Moderate to advanced	Flap surgery per quadrant	Rs 4,000 - 18,000	Rs 2,000
Advanced with bone loss	Flap surgery with bone graft	Rs 8,000 - 50,000	Rs 3,000

Notice the shape of that table. The cost of treating gum disease rises by an order of magnitude at each stage, and the earliest stage is the cheapest procedure in dentistry. Nothing else in this book has that ratio.

The maintenance interval nobody follows

After treatment, periodontitis needs maintenance cleaning every three to six months, not once a year, because the pockets that formed do not close completely. Patients who keep that interval keep their teeth. Patients who return when something hurts usually do not.

> **Smoking, in one paragraph**
> Smoking narrows the blood vessels in the gum, which masks the bleeding that would otherwise warn you. Smokers often have worse gum disease and less bleeding, which is why they discover it later. If you smoke and your gums do not bleed, that is not good news.

CHAPTER 18

Children from zero to fourteen

The visit schedule, the four procedures worth paying for, and the myths that cost parents money.

Children's dentistry is the highest-return part of the whole field, because almost everything that happens to an adult's teeth started in a child's mouth. It is also where Indian parents are most often sold things they do not need, and skip things they do.

Age	What matters	What to do
0 - 2	First tooth appears around 6 months; bottle and breastfeeding at night cause decay	Wipe gums; first dental visit by the first birthday; no bottle in bed
3 - 5	Milk teeth cavities spread fast; thumb sucking and dummies affect the bite	Supervised brushing with a pea of fluoride paste; treat cavities, do not wait for the tooth to fall out
6 - 8	First permanent molars arrive with deep grooves	Fissure sealants and fluoride varnish; teach flossing
9 - 12	Mixed dentition; crowding becomes visible	Orthodontic assessment by age 7 if not already done; mouthguard for contact sport
13 - 14	All permanent teeth except wisdom teeth are in	Full orthodontic assessment; address tobacco and energy drinks

The four things worth paying for

- Fluoride varnish, two to four times a year for high-risk children. Cochrane's pooled trials show a 43 per cent reduction in decayed, missing and filled surfaces in permanent teeth and 37 per cent in milk teeth. At Rs 500 to Rs 3,000 a session, it is the best-value preventive purchase in dentistry.
- Fissure sealants on the first permanent molars, applied as soon as the tooth erupts fully. Cochrane's review found resin sealants reduced cavities by 11 to 51 per cent at 24 months compared with no sealant.
- A treated milk tooth. A milk molar that is left to decay until it falls out can abscess, damage the permanent tooth underneath, and lose space, which then needs a space maintainer at Rs 3,400 to Rs 4,000 on the CGHS list.
- A mouthguard for contact sport. A custom guard costs a fraction of a re-implanted front tooth, and front teeth are the ones that break.

Three myths that cost money

- 'Milk teeth will fall out anyway.' They fall out on a schedule, and an infected one can damage the tooth that replaces it. It also hurts, which children do not always report.
- 'Braces should wait until all the teeth are in.' Some problems, like a crossbite or a severe overbite, are much easier to fix during growth. That is why the first assessment is at seven, not fourteen.
- 'A child is too young for the dentist.' The first visit by the first birthday is a conversation and a look, not a drill. It builds the habit while it is still free.

> **The 30-second nightly check**
>
> Stand at the bathroom door while your child brushes and watch for three things: are they reaching the back teeth, are they spitting rather than swallowing, and are the gums bleeding. Three observations, thirty seconds, and it catches most of what goes wrong.

CHAPTER 19

Teenagers: wisdom teeth, tobacco and the three-week rule

The three decisions that land in a 16 to 22-year-old's mouth, and how to make them.

Wisdom teeth

Not every wisdom tooth needs to come out. The United Kingdom's NICE guidance is the clearest published position: asymptomatic third molars with no disease should not be removed prophylactically, because the risks of surgery outweigh the benefit. Removal is indicated when there is repeated infection around the tooth (pericoronitis), decay that cannot be treated, a cyst, damage to the adjacent tooth, or a tooth that is preventing a necessary orthodontic movement.

What that means in practice: a wisdom tooth that has never caused a problem and is not visible on an X-ray as damaging its neighbour can be watched. One that has caused two episodes of swelling in a year should come out, and the cost rises sharply with impaction. A soft-tissue impacted tooth sits at Rs 1,700 to Rs 2,000 on the CGHS list and Rs 2,500 to Rs 18,000 privately; a bony impacted one is Rs 4,250 to Rs 5,000 on the CGHS list.

Tobacco, gutka and vaping

Tobacco use among Indians aged 15 and over is 28.1 per cent, according to the WHO country profile. Smokeless tobacco, in the forms sold as gutka, khaini, zarda, pan masala with tobacco and mishri, is the dominant driver of oral cancer in India, and it causes changes in the mouth long before cancer: white patches (leukoplakia), red patches, and oral submucous fibrosis, which stiffens the cheek until opening the mouth becomes difficult.

Oral submucous fibrosis is not cancer and it does not reverse. It is treated by stopping the habit and, in advanced cases, surgery to release the fibrous bands, which is a PM-JAY covered procedure. The message to a teenager is simple and does not need softening: this is the one habit that can take the ability to open your mouth.

The three-week rule

Any ulcer, white patch, red patch or lump in the mouth that has not healed in three weeks needs an examination, not a mouthwash

Most mouth ulcers heal in 7 to 14 days. One that persists past three weeks, especially under the tongue, on the side of the tongue, or on the floor of the mouth, needs to be looked at by a dentist or an oral medicine specialist. Oral cancer is highly treatable when found early and much harder when found late. India recorded an estimated 135,929 new cases of lip and oral cavity cancer in 2020, with an incidence rate of 9.8 per 100,000.

A dental visit is a cancer screening whether or not anyone calls it that. Ask your dentist to check the soft tissues, the tongue borders and the floor of the mouth at every examination. It takes twenty seconds.

CHAPTER 20

Pregnancy and your teeth

What changes, what is safe, and the one timing rule that matters.

Pregnancy does not cause tooth decay, and the old saying about losing a tooth per child is wrong. What pregnancy does is change the environment: hormones make the gums more reactive to plaque, vomiting exposes enamel to acid, and cravings for sweet food often increase. Gingivitis in pregnancy is common and reversible.

- Tell your dentist you are pregnant at the first visit, before any X-ray or prescription.
- Routine treatment, including fillings and scaling, is generally safe. The comfortable window for elective work is the second trimester, but urgent treatment for pain or infection should not be postponed in any trimester.
- Dental X-rays with a lead apron and modern digital sensors deliver a very small dose. If a film is needed to diagnose pain, the risk of an untreated infection is the larger one.
- Morning sickness: rinse with plain water after vomiting and wait 30 minutes before brushing, so you are not scrubbing softened enamel.
- Gum care is not cosmetic in pregnancy. Untreated periodontal infection has been associated with adverse pregnancy outcomes in observational studies, and the treatment is a cleaning.

The one appointment to book

A cleaning and a check-up in the second trimester, with a note of the due date. It costs the price of a cleaning and removes the most common reason a new mother ends up in a dental chair in pain six weeks after delivery, when leaving the house is hardest.

CHAPTER 21

Diabetes, heart disease and your mouth

Why your dentist asks about your sugar, and what the connection is worth in practice.

The link between gum disease and diabetes runs in both directions, and it is the best-established mouth-to-body connection in medicine. High blood sugar worsens gum infection; severe gum inflammation makes blood sugar harder to control. Both statements are supported by a large body of observational and interventional work.

- If you have diabetes, your risk of periodontitis is roughly two to three times higher, and healing after extraction or implant surgery is slower.
- Treating gum disease produces a small but real improvement in blood sugar control. Meta-analyses put the average reduction in HbA1c at around 0.3 to 0.4 percentage points. That is not a substitute for medication; it is a meaningful addition to it.
- Uncontrolled diabetes is a reason to postpone elective implant surgery. It is not a reason to postpone treating an infection.
- Dry mouth is common in diabetes and in many medicines, and dry mouth is a direct cause of root decay.

The heart question

Dentists ask about heart conditions for two reasons: bleeding risk with blood thinners, and the small group of patients who need antibiotics before dental work. That group has narrowed sharply. Current guidance limits antibiotic prophylaxis to patients with specific cardiac conditions, such as a prosthetic heart valve, a previous infective endocarditis, or certain congenital heart defects. A mitral valve prolapse or a stent is generally not on that list. If a clinic hands you antibiotics before a cleaning without asking about your cardiac history, ask why.

> **What to bring to a dental visit if you have a chronic condition**
> A list of your medicines with doses, your latest HbA1c or blood pressure reading, the name of the doctor who manages the condition, and your last INR if you take warfarin. Four items, and it changes the plan.

CHAPTER 22

After sixty: dry mouth, root decay and the denture decision

The four problems that define dental care in later life, and what each one costs.

The WHO country profile puts complete tooth loss at 4 per cent of Indians over 20, and the proportion rises steeply with age: a large central India survey found complete edentulism in about 12 to 13 per cent of people aged 60 and above. Most people over 60 still have teeth, and the goal is to keep them.

1. Dry mouth is the main enemy

Saliva is the body's own protective film. When it drops, decay accelerates, especially at the neck of the tooth where there is no enamel. The usual culprits are blood pressure medicines, antidepressants, antihistamines, diuretics and diabetes. Ask your physician whether any of your medicines can be changed; if not, manage it: sip water, use a fluoride toothpaste and a fluoride mouthwash, and ask your dentist about high-fluoride varnish.

2. Root caries

Decay at the gum line in older adults is often painless and progresses around the root, sometimes under a filling or a crown, until the tooth fractures. This is why the six-month check in later life is not a formality.

3. Dentures versus implants

Option	Private range	CGHS benchmark	Consider when
Complete denture, per arch	Rs 8,000 - 30,000	Rs 10,000	Bone loss is severe, budget is fixed, surgery is risky
Removable partial denture	Rs 2,000 - 8,000	Rs 2,000 - Rs 4,000	A few teeth missing, adjacent teeth sound
Implant-supported overdenture	Rs 45,000 - 3,50,000	Not listed	Loose lower denture, adequate bone, good health
All-on-4 or All-on-6 fixed arch	Rs 1,50,000 - 7,00,000	Not listed	Full arch failure, high budget, committed hygiene

A well-made denture is a good solution. A cheap denture that moves is the reason many older patients stop eating properly, and the nutritional cost of that is larger than the dental one. If a lower denture will not stay, two implants to anchor it can transform the fit for a fraction of a full-arch case.

4. The practical problem nobody plans for

Arthritis, tremor and reduced grip make brushing hard. An electric brush with a large handle, a long-handled interdental brush, and a chair to sit on while brushing solve most of it. If a parent or grandparent is being cared for at home, add a daily mouth check to the routine, because decay and denture sores in a dependent adult are usually found late.

The number to budget

For an adult over 60 with natural teeth: one cleaning and check-up a year, one X-ray every two to three years, and an assumption of one crown or one extraction every three to five years. In metro private prices that is roughly Rs 5,000 to Rs 12,000 a year on average, and considerably less on a CGHS panel or at a dental college.

CHAPTER 23

Tobacco, gutka and oral cancer

The one chapter in this book that is not about money.

India recorded an estimated 135,929 new cases of lip and oral cavity cancer in 2020, an incidence rate of 9.8 per 100,000, according to the WHO's country profile. A 2026 review of India's oral health system cites a higher 2022 figure of 143,759 new cases. Either number makes this one of the most common cancers in Indian men, and it is largely preventable.

Tobacco use among people aged 15 and over is 28.1 per cent nationally. The forms that matter most for oral cancer are the smokeless ones: gutka, khaini, zarda, pan masala containing tobacco, and mishri. They are held in the cheek for minutes at a time, which is precisely the exposure pattern that produces cancer at the site of contact.

What to look for

- A white patch that cannot be rubbed off (leukoplakia) or a red patch (erythroplakia).
- An ulcer that has not healed in three weeks, particularly on the side or underside of the tongue, the floor of the mouth, or the inside of the cheek.
- A lump in the cheek or neck, or a feeling that something is caught in the throat.
- Difficulty opening the mouth, or a burning sensation with spicy food, which can be early oral submucous fibrosis.
- Loose teeth without gum disease, or numbness in the lip or chin.

What happens if something is found

A dentist will examine and, if needed, refer for a biopsy, which the CGHS list prices at Rs 1,200 to Rs 2,000. Early oral cancer is treated with surgery, and surgery for oral cancer is covered under Ayushman Bharat PM-JAY for eligible families. Late-stage treatment is disfiguring, expensive and far less successful. A study of the economic burden of oral disease in India found out-of-pocket costs for oral cancer treatment ranging from Rs 80,000 to over Rs 2,30,000.

The habit that halves the risk

Stopping. Not reducing, not switching to a 'safer' brand, not moving to vaping as a permanent substitute. Ask your dentist or a government tobacco cessation centre, which the National Oral Health Programme runs alongside its screening work, for help. The first two weeks are the hard part and it is a chemical problem, not a willpower problem.

PART FOUR

Emergencies, procedures and the market

What to do in the first hour, the first night and the first week, and how to read the offers you will be sold

CHAPTER 24

A tooth knocked out: the first sixty minutes

The one page in this book worth memorising. It is the difference between keeping a front tooth and paying for an implant.

A knocked-out permanent tooth is one of the few true emergencies in dentistry. The International Association of Dental Traumatology's 2020 guidelines are unambiguous: replant the tooth at the scene if you possibly can, because the cells on the root surface start to die within minutes of drying.

1. Check it is a permanent tooth. Baby teeth are never replanted, because replanting can damage the developing permanent tooth underneath.
2. Pick the tooth up by the crown, the white part. Never touch the root.
3. If it is dirty, rinse it briefly with milk, saline or the patient's own saliva. Do not scrub it and do not use water if you have anything better.
4. Put it straight back into the socket, the right way round, and press gently. Ask the person to bite on a clean handkerchief or gauze to hold it in place.
5. If you cannot replant it, put it in milk. Milk is the best household medium. Saliva or saline are acceptable; water is poor but better than air. Do not wrap it in tissue.
6. Get to a dentist immediately. Call ahead so they are ready.

The prognosis bands from the guideline are worth knowing because they explain the urgency: cells are most likely viable if the tooth is replanted within about 15 minutes; viable but compromised if it is stored in milk and the total dry time is under 60 minutes; likely non-viable after 60 minutes of dry time. Even then, replantation is almost always the right decision, because it preserves bone and keeps future options open.

At the clinic, expect local anaesthesia, a flexible splint for two weeks, a tetanus check, antibiotics, and root canal treatment within two weeks for a mature tooth. Follow-up continues for years, because a replanted tooth can ankylose or resorb later.

> **Put this in your phone notes now**
> Knocked-out permanent tooth: handle by the crown, rinse in milk, replant immediately, bite on cloth, go straight to a dentist. Milk, not water. Never scrub. Never wrap in tissue.

CHAPTER 25

Severe pain at two in the morning

What actually helps, what makes it worse, and the four signs that mean hospital, not clinic.

Dental pain peaks at night for a physiological reason: lying down increases blood pressure in the tooth, and there is nothing to distract you. The pain is real and it is not a reason to wait for morning if there is swelling.

What helps

- A painkiller at the dose on the label. Paracetamol or ibuprofen, not both at once unless a doctor has told you to, and never on an empty stomach. Ibuprofen is not for everyone: check with a pharmacist if you have stomach ulcers, kidney disease, asthma or are pregnant.
- Cold compress on the outside of the cheek for 15 minutes at a time, for swelling.
- Sleep with your head raised on two pillows.
- Warm salt water rinse, if the pain is from a sore gum rather than a deep cavity.
- Clove oil on a cotton bud held against the tooth for a few minutes. It is a traditional anaesthetic and it does work, briefly. Do not swallow it.

What makes it worse

- Putting an aspirin or a painkiller tablet directly on the gum. It burns the tissue and creates a chemical ulcer on top of the toothache.
- A hot compress on a swollen face. Heat spreads infection.
- Antibiotics borrowed from someone else. They do not treat the cause, they mask it, and they contribute to resistance.
- Waiting for the pain to disappear on its own. When it does, the nerve has usually died, and the infection is still there.

The four signs that mean hospital, now

1. Swelling that reaches the eye or closes it.
2. Swelling under the jaw or in the floor of the mouth.
3. Difficulty swallowing, or a change in your voice.
4. Difficulty breathing, or a high fever with the swelling.

Those four are deep space infections. They can compromise the airway and they are treated with intravenous antibiotics and surgical drainage, not with a tablet and an appointment on Thursday.

> **What to say on the phone at 11 p.m.**
> 'I have severe dental pain with swelling on the left side of my face, it started two days ago, and I have a fever of 101. Can you see me tonight, or should I go to a hospital emergency?' That one sentence gets a triage decision instead of a receptionist reading a schedule.

CHAPTER 26

After the chair: recovery, and the signs that mean go back

Recovery timelines for the five common procedures, and the specific warning signs for each.

Procedure	Normal for how long	Go back if
Simple extraction	Bleeding stops in 30-60 min; soreness 2-3 days	Pain and a bad taste worsen on day 3-4 (dry socket); bleeding restarts heavily
Surgical or impacted extraction	Swelling peaks at 48-72 hours; jaw stiffness 5-7 days	Swelling grows after day 3, fever, numbness that does not fade
Root canal	Tenderness on biting for 2-3 days	Pain increases after day 4, or the temporary filling comes out
Deep cleaning	Sensitivity to cold and slight bleeding for 1-2 weeks	Bleeding continues past two weeks, or a tooth becomes mobile
Implant placement	Swelling 2-3 days, mild ache 5-7 days	Discharge, fever, or the implant feels loose

The first 24 hours after an extraction

- Bite firmly on the gauze for 30 to 45 minutes. Do not keep changing it every five minutes, which restarts the clot.
- No smoking, no straws, no vigorous rinsing, no spitting. All four dislodge the blood clot, and a lost clot is a dry socket, which is more painful than the extraction was.
- No hot food or drink on day one. Soft, cool food.
- Start gentle salt water rinses from the second day, not the first.
- Sleep with your head slightly raised on the first night.

Dry socket deserves a sentence of its own. It happens when the clot is lost, usually on day three or four, and it produces a deep, throbbing pain and a bad taste. It is not dangerous but it needs a dressing at the clinic, and it is the most common reason people return after an extraction. Smoking in the first 48 hours sharply raises the risk.

The question to ask before you leave the chair

'What is normal for the next three days, and what number do I call if it is not?' Write the number down. A clinic that has no after-hours number should tell you which hospital to use instead.

CHAPTER 27

Antibiotics and the toothache that will not settle

Why the tablet is not the treatment, when it genuinely is, and why this matters beyond your mouth.

A toothache is mostly a mechanical and inflammatory problem. A cavity exposes the nerve; the nerve becomes inflamed; pressure builds inside a closed space. Antibiotics do not reach the inside of a dead tooth, because there is no blood supply there to carry them. The definitive treatment is removing the infected tissue, which means a root canal or an extraction.

When antibiotics are genuinely needed

- Spreading infection: facial swelling, fever, swelling that tracks towards the eye or neck.
- Systemic involvement, or infection in someone immunocompromised, diabetic and uncontrolled, or on chemotherapy.
- Certain surgical situations, as prescribed after the procedure.
- Not for: a tooth that hurts but has no swelling, a routine extraction, or a cavity that has not yet infected anything.

Why overuse is your problem too

India is one of the largest consumers of antibiotics in the world, and dental prescriptions are a meaningful share of outpatient use. Every unnecessary course adds to resistance that will matter to you or your children in a hospital one day. It also creates a false sense of resolution: the pain settles, the patient does not return, the tooth continues to decay, and the eventual treatment is an extraction plus an implant instead of a filling.

The honest sequence

1. Pain relief, at the correct dose, for the hours until you are seen.
2. Diagnosis, with an X-ray and a vitality test.
3. Definitive treatment: filling, root canal, or extraction.
4. Antibiotics only if the infection has spread beyond the tooth, and then as an adjunct to treatment, not instead of it.

If a clinic hands you antibiotics and no appointment

Ask one question: 'When will the tooth itself be treated?' A course of tablets without a plan for the tooth is a delay, not a treatment, and the delay is what turns a Rs 6,000 root canal into a Rs 40,000 implant.

CHAPTER 28

Chains, dental tourism and the free check-up

How to read the three offers you will meet, and the five things to verify before you pay anyone.

The free or Rs 99 check-up

It is a legitimate marketing model, and it is worth using if you use it correctly. The consultation is free; the treatment plan that follows is where the clinic earns. Go, accept the X-ray, take the written plan away, and compare it with the price tables in chapter 2 before you commit to anything above Rs 5,000. The discount offered for same-day payment is a sales device, not a clinical one.

The chain clinic

Chains bring standardised materials, audited sterilisation and predictable pricing, and they cost 20 to 50 per cent more than an independent clinic for the same procedure. That premium buys consistency, not necessarily better clinical judgement. Ask who the treating dentist is, whether specialists are on site, and what the warranty covers if the crown fails in another city.

Dental tourism

Treatment abroad or in another state can be genuinely cheaper for large work: implants, full-mouth rehabilitation, orthodontics. The risks are not the surgery, they are the follow-up. A crown that needs adjusting in month four, an implant that needs review, or a root canal that flares up cannot be handled by phone. If you go, insist on a written treatment plan with material brands before you travel, a copy of all records and X-rays, and a named dentist in your own city who has agreed in advance to handle follow-up.

Five things to verify before you pay anyone

1. Registration. Ask for the dentist's registration number and the state dental council it is with. The Dental Council of India publishes state-wise registration counts, and state councils can confirm an individual.
2. Qualification for the procedure. An impacted wisdom tooth, a re-treatment, an implant and orthodontics are specialist procedures. BDS is the licence; MDS in the relevant speciality is the training.
3. Sterilisation, visibly. Instruments should come out of a sealed pouch in front of you. Ask.
4. Written plan with materials named. 'Zirconia' is a category, not a brand.
5. Warranty in writing, with the conditions. What voids it, who honours it, and for how long.

> **The one offer to refuse**
>
> A large treatment plan with a discount that expires in 48 hours. Real clinical urgency has a reason you can understand: swelling, infection, a fracture. A deadline is a sales technique, and it is the clearest signal in this chapter.

PART FIVE

The worksheets

Five pages to print, fill in and keep. They are the part of this book you will still be using in three years.

CHAPTER 29

Worksheet 1: the annual dental budget

Fill this once a year. Most families overspend on dental care because they only ever spend in emergencies.

Dental costs are predictable in aggregate even when they are unpredictable in detail. Every adult needs a cleaning a year, most need one restorative procedure every few years, and every child needs preventive care. Budgeting for the average stops a single Rs 30,000 surprise from becoming a loan.

Line	Who	When	Budget (Rs)	Actual (Rs)
Check-up and cleaning	Every adult	Yearly	1,000 - 2,500	
Check-up	Every child	6-monthly	600 - 2,000	
Fluoride varnish	Children at risk	2-4 times a year	1,000 - 12,000	
Filling, if needed	Adults	As needed	800 - 3,200	
Crown or root canal reserve	Adults	Every 3-5 years	9,000 - 25,000	
Orthodontic reserve	Children 10-14	Once	35,000 - 90,000	
Denture or implant reserve	Adults over 55	Once	8,000 - 75,000	
Emergency fund	Family	Always	10,000	
Total for the year				

The rule of thumb

Put aside roughly Rs 150 a month per adult and Rs 200 a month per child, from age 30. By the time a crown is needed, the money exists, and the decision stops being driven by what you can afford this month.

Notes for the year

Date	What was done	Clinic	Cost (Rs)	Notes

CHAPTER 30

Worksheet 2: the quote comparison sheet

Two clinics, side by side, on the same line items. This is the page that finds the Rs 6,000 difference.

A quote is only comparable if you compare the same things. Ask both clinics to fill the same columns, or fill them yourself from the written plans, and the decision usually makes itself.

Item	Clinic A	Clinic B	Included?
Procedure and tooth number			
Diagnosis given, in words			
X-ray or scan provided			
Number of visits			
Anaesthesia included			
Crown included (which material, which brand)			

Item	Clinic A	Clinic B	Included?
Post and core included			
Laboratory charges included			
Medicines included			
Review visits included			
Warranty period and terms			
Payment terms and EMI			
Total, all in			

How to read the finished sheet

Compare the last row only after reading every row above it. If Clinic A is Rs 4,000 cheaper and Clinic B includes the crown, the lab charges and two reviews, Clinic A is not cheaper. It is incomplete.

CHAPTER 31

Worksheet 3: the second opinion request

Copy this. Send it on WhatsApp or say it at the desk. It works because it is polite and specific.

The message

Copy and send

Hello, I have a treatment plan from another clinic for a root canal on my lower right molar, and I would like a second opinion before I start. I can bring the written plan, the X-ray and a photo. I am not asking anyone to criticise the first clinic, I would just like to know whether you would treat it the same way and what you would do differently. Could I get a 20-minute appointment this week? Thank you.

What to take

Item	Have it?	Notes

What to record at the appointment

Question	Clinic A said	Second opinion said

The decision rule

If both dentists agree on the diagnosis and differ only on materials, choose on price. If they disagree on the diagnosis, get a third opinion from a dental college. Never choose on price when the diagnosis itself is in dispute.

CHAPTER 32

Worksheet 4: the children's tracker

One page per child. Six-monthly check-ups, brushing ticks, and the dates that matter.

Child's name	Date of birth
First dental visit	Sealants done (which teeth)
Fluoride varnish dates	Orthodontic assessment done
Last check-up	Next check-up due

Brushing log, one month

Week	Morning ticks	Night ticks	Bleeding?	Notes

The three numbers for a child

First visit by the first birthday. A check-up every six months. Braces assessed by age seven, treated around eleven to fourteen if needed. Everything else in children's dentistry is a detail hanging off those three.

CHAPTER 33

Worksheet 5: your appointment notes

Take this to every visit. Five minutes of notes saves thousands of rupees over a decade.

Date	Clinic and dentist	Complaint	Diagnosis	Plan and cost	Next step

The standing list to carry

- Your medicines, with doses.
- Any allergy, especially to penicillin or latex.
- Your last HbA1c and blood pressure, if you have diabetes or hypertension.
- Whether you take a blood thinner, and which one.
- Whether you are pregnant or could be.
- Your insurer's helpline number and policy number, if you have dental cover through work.

The habit that pays for this book many times over

Keep every X-ray, every plan and every bill in one folder, on your phone. When you change cities, change clinics, or need a second opinion, you start with evidence instead of memory. It takes ten seconds at the desk and it is the difference between a diagnosis and a guess.

APPENDIX

Sources, dates and how to check them

Every figure in this playbook traces to one of these. Where a number is a range, it is because the sources disagree, and the disagreement is stated in the text.

Official and government sources

- World Health Organization, India Oral Health Country Profile, 2022 (WHO/UCN/NCD/MND/IND/2022.1). Dentists 271,744 in 2019; untreated caries in deciduous teeth of children aged 1-9 at 43.3%; severe periodontal disease at 21.8% of those aged 15+; edentulism 4.0% at 20+; 135,929 new lip and oral cavity cancer cases in 2020 at 9.8 per 100,000; tobacco use 28.1% at 15+; sugar availability 53.8 g per person per day; per-capita dental expenditure USD 0.05; productivity losses USD 7,249 million; health benefit package coverage of routine and essential curative care yes, advanced curative and rehabilitation no.
- Central Government Health Scheme, procedure rate list revised 13 October 2025 (CGHS OM No. 5-16/CGHS(HQ)/HEC/2024, Part I, dated 03.10.2025), Tier I rates. All CGHS figures in this book are from the Non-NABH and NABH columns of that list.
- Employees' State Insurance Corporation circular adopting the CGHS rate list across ESI user locations, effective 1 December 2025.
- National Health Authority, Health Benefit Package 2.2 manual (nha.gov.in). The dental exclusion clause quoted in chapter 3 is from the exclusions annexure of that document; oral and maxillofacial surgery is covered as a specialty package group.
- Press Information Bureau, release of 19 August 2025 on the AIIMS Bhopal oral health survey of Madhya Pradesh: 47,837 participants across 41 districts; dental caries 40-70%; gum disease 50-87%; precancerous lesions 2-17%; caries above 70% in the elderly and in nearly half of five-year-olds.

Peer-reviewed sources

- Oral health status and oral healthcare system in India: a narrative review, International Journal of Community Medicine and Public Health, 2026. Pooled gingivitis 46.6% and periodontitis 51% at 15+; approximately 3.7 lakh dentists; rural availability about 1:25,000; no national oral health survey since 2002-03.
- Economic burden and economic impact of oral diseases in India: a systematic review and meta-analysis, 2026 (PMC12856868). Insurance coverage below 15%; catastrophic health expenditure pooled at 18.8%; up to one-third of patients using borrowing or asset sales; oral cancer out-of-pocket costs Rs 80,000 to over Rs 2,30,000.
- Fouad A F et al., International Association of Dental Traumatology guidelines for the management of traumatic dental injuries: 2. Avulsion of permanent teeth, Dental Traumatology, 2020. Replantation at the scene, storage media in order of preference, the 15-minute and 60-minute prognosis bands, two-week flexible splinting, root canal within two weeks.
- Cochrane Oral Health, Fluoride varnishes for preventing dental caries in children and adolescents (2013): 43% reduction in decayed, missing and filled surfaces in permanent teeth, 37% in primary teeth, moderate-quality evidence.
- Cochrane Oral Health, Sealants for preventing tooth decay in permanent teeth (2017): resin-based sealants reduced caries by 11-51% at 24 months compared with no sealant, moderate-quality evidence.
- Cochrane Oral Health, Sealants versus fluoride varnishes (2020): no reliable evidence that one is superior; both are effective.

Market and price sources

- Observer Research Foundation, Dentistry in India: oversupply and gender bias shaping oral health workforce, 21 November 2025. 376,721 dentists registered with the Dental Council of India; one dentist per 3,846 people; 329 dental colleges with 27,926 BDS seats, 82% in private institutions; the National Dental Commission Act, 2023.
- Dental Council of India, state-wise registered dentist counts (dciindia.gov.in).

- Published 2026 private clinic price lists used for the metro and tier-2 ranges in chapter 2: Delhi NCR (Greater Kailash and East Delhi clinics), Hyderabad, Lucknow, Indore and Patna, plus a national range page that aggregates clinic data. These are indicative published ranges, not quotes, and they move with locality and case complexity.

How to check anything in this book yourself

- For CGHS rates: cghs.gov.in, procedure and investigation lists, and confirm the revision date on the document.
- For PM-JAY coverage: pmjay.gov.in and the NHA Health Benefit Package master; search the package name and read the exclusion annexure.
- For your dentist's registration: the state dental council, or the DCI registry pages.
- For a clinic's empanelment: the CGHS, ESIC or PM-JAY hospital locator, filtered by speciality and district.
- For anything clinical: ask the treating dentist to show you the evidence on your own X-ray. A number in a book, including this one, is a benchmark. Your film is the fact.

> **One last thing**
>
> This playbook is educational. It is not a diagnosis, it does not create a doctor-patient relationship, and it cannot see your mouth. Use it to ask better questions, to recognise a plan that is too aggressive, and to know what things cost. The person who treats you should be a registered dentist who has examined you, shown you the evidence, and put the plan in writing.